



Havana Specialist Hospital, A Guide for Board Members and Committee Chairs

An orientation manual for the executive directors

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Companion to: the HSH Board Operating Model

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This guide is written for directors serving on a formal board for the first time, and for those chairing a board committee for the first time. It expands the Operating Model into a plain-language manual: what the board does, what is expected of you, how much work it really is, how your role as an executive director fits alongside your day job, and how to chair a committee well. Read it once end to end, then keep it as a reference. Nobody is expected to know all of this on day one. Learning it is exactly what the first year is for.

1. How to read this guide, and a word of reassurance

Sitting on a board for the first time can feel like being handed a cockpit. There are new words, new meetings, new responsibilities, and a sense that everyone else already knows the rules. They do not. Almost every good director started exactly where you are now, and learned it by doing it, with support.

Two things are true at once, and this guide holds both honestly:

- **This is real work.** Being a director of a hospital, and chairing a committee, carries genuine responsibility and asks for genuine time and attention. We will not pretend otherwise.
- **It is entirely learnable, and you are well supported.** You have an experienced chair, a Company Secretary to run the administration, trusted fellow directors, and a structured enablement and training programme designed to build exactly the knowledge this guide describes.

If you take one idea from this manual, take this: *a board does not run the hospital day to day. It sets the direction, checks that the direction is being followed, and steps in when something is off course.* Everything else follows from that.

2. The single biggest shift: governing is not managing

This is the idea that trips up almost every new director, especially able people who are used to getting things done themselves.

Management runs the hospital. The board governs it. These are different jobs, and the board's value comes precisely from *not* doing management's job.

- **Managing** is daily: running clinics, staffing shifts, chasing payments, fixing the things that break today. At HSH, this is done by management and the Afya team, under the Medical Director.



- **Governing** is periodic and elevated: agreeing where the hospital is going, approving the budget and the big decisions, watching performance and risk, holding management to account, and protecting the interests of the owners. This is your job as a director.

A useful test when you are unsure which hat you are wearing: *am I about to make or take over a management decision, or am I setting direction, approving, or holding someone to account?* The first is management's job. The second is yours.

New directors often feel they should "help" by getting involved in operations. Resist it. When the board reaches down into management, three things happen: management loses ownership, the board loses the independence it needs to judge management fairly, and nobody is clearly accountable. The most disciplined thing a director can do is govern well and let management manage.

3. Your two hats: being an executive director and a board member

You are an **executive director**, which means you hold two roles at once, and it is worth being precise about them because they can pull in different directions.

- As an **owner and executive director**, you have a stake in the hospital and a title that reflects your standing in it.
- As a **member of the board**, you have a legal and fiduciary duty to act in the best interests of *the company as a whole*, both founding families together, not one family, not one department, and not your own preference.

When you sit in a board or committee meeting, the second hat is the one you wear. This is the discipline that makes a board trustworthy: in the room, you are a steward of the whole institution. Outside the room you may be an owner with your own interests, and that is legitimate, but board decisions are taken for the company.

What this means in practice:

- You speak and vote for what is best for HSH, even when it is not what is most convenient for you or your side of the family.
- You keep board discussions confidential.
- You declare any personal or family interest in a matter and step out of that decision (more in section 10).
- You support a decision once the board has made it properly, even if you argued against it, because a board that cannot commit to its own decisions cannot govern.

This dual role is normal and workable. Millions of family-business directors do it every day. The key is simply to know which hat you are wearing, and in the boardroom, it is always the board hat.

4. A special case worth understanding: the Medical Director

Dr Ijeoma Iwuala is both the **Medical Director** and an **executive director**. Her situation is the clearest example of the two-hats idea, and understanding it helps everyone understand the whole model.

In reality, her role plays out like this:



- As **Medical Director**, she carries genuine, daily management responsibility. She runs clinical operations, leads the clinical team, and is accountable for how the hospital actually performs on the ground. This is a full executive job on its own.
- As an **executive director** on the board, she is also one of the eight people who govern the hospital.

Here is the important part, and it is a feature of good governance, not a slight: **the Medical Director reports *in* to the board and its committees; she does not chair them.** She will not chair any committee. There are three plain reasons, and they protect her as much as the board:

1. **You cannot assure your own work.** The board's committees exist to review and challenge how the hospital is run, including how it is run clinically and operationally. Since the Medical Director *is* the person running much of that, she cannot also be the person independently judging it. That would be marking her own homework, and it would leave the board with no genuine check.
2. **Her executive job already fills her hands.** Running the hospital day to day is demanding and important. Adding the work of chairing a board committee on top would either dilute her operating role or the committee's oversight, or both.
3. **The board must be free to challenge management, including her.** If the Medical Director chaired the very committees meant to hold management to account, other directors would find it harder to question freely. Keeping her as the executive who reports in, rather than the chair who presides, keeps that challenge healthy.

So what is her board role? It is substantial and respected. She sits on the **Quality and Clinical Governance Committee** as the *executive clinical lead*, the person who brings the clinical reality into the room, presents the quality and safety picture, answers the committee's questions, and carries its decisions back into operations. She is the bridge between how care is actually delivered and how the board assures it. That is a position of real influence. It is simply a different position from chairing.

The same logic applies, in lighter form, to any director who also carries executive management responsibility: where you help run something, you should not be the one independently assuring it. This is why the committees that oversee operations and finance are chaired and co-chaired by directors who are not running those operations day to day, and why they oversee the performance delivered by Afya and management rather than their own line work.

5. How you steer a whole hospital in four meetings a year

This is the question that gives new directors pause, and it deserves a direct answer: *how can a board possibly direct a complex organisation, with all its moving parts, meeting only once a quarter?*

The answer is that the board is not trying to drive the car minute to minute. It is setting the destination, reading the instruments, and correcting the course at intervals. A large ship is not steered by wrestling the wheel every second; it is steered by setting a heading, watching the dials, and making considered adjustments. Your quarterly board meeting is that considered adjustment.

This works because of three things, and understanding them removes most of the worry:

1. **Management runs the moving parts between meetings.** The hundreds of daily decisions are made by management and Afya, within the authority the board has delegated to them (the Delegation of Authority schedule). You are not expected to be involved in those. You are expected to have set the limits within which they are made.



2. **Good reporting brings the organisation to you.** You do not have to be everywhere. A well-built board pack, arriving before each meeting, tells you what happened, what is off target, what the risks are, and what decisions you need to make. Reporting is how a quarterly board stays in control of a daily organisation. This is why the templates and the reporting discipline matter so much, they are the instruments on your dashboard.
3. **The committees do the deeper work between board meetings.** The board meets quarterly, but its committees meet more often and go deeper into finance, quality, risk, and so on. By the time a matter reaches the full board, a committee has usually already examined it in detail. The board relies on its committees so that no single meeting has to do everything.

So the honest picture is this: the *board* meets quarterly to direct and decide, but the *governance system*, committees, reporting, and delegated management, works continuously underneath it. You are directing a great organisation with many moving parts precisely by building and trusting that system, not by attending every moment of it.

Two things follow for you as a director. First, because you meet only quarterly, **each board meeting carries weight**, so preparing properly for it matters more, not less. Second, because you rely on reporting, **you must learn to read it and to ask good questions of it**. Those two skills, preparation and questioning, are most of what makes a good director.

6. What is actually expected of you: an honest picture of the work

Let us be straight about the commitment, because a clear picture is less daunting than a vague worry. Being an executive director and a board member is not a ceremonial honour and it is not a walk in the park. It is a real, ongoing responsibility that sits *alongside* whatever else you do. Here is what it asks.

6.1 Your time, realistically

Commitment	Roughly how often	What it involves
Full board meetings	4 a year (quarterly), plus a strategy session	Half a day each in the room, plus two to four hours reading the pack beforehand
Committee meetings	Depends on your committee, from twice a year (RemCo) to quarterly (most) to monthly during active phases (Transformation)	An hour or two each, plus preparation. Chairs also prepare the agenda and confer with management and the Company Secretary
Between-meeting duties	Ongoing, light but real	Reading, responding to occasional approvals, staying informed, the odd ad hoc matter
Your executive role	As it already is	This board work is <i>in addition</i> to your existing responsibilities

Totalled up, most directors here should plan for the equivalent of roughly one to two days of board and committee work per month once things are running, more in the busy early months while the model beds in, and more again if you chair a committee. That is a real commitment, and it is also a manageable one for people who plan for it.

6.2 What you are expected to know



You are not expected to be an expert in everything. You *are* expected, over your first months, to build a working understanding of:

- **The business.** How HSH makes and spends money, its main services, and what drives its performance.
- **The numbers.** Enough to read the management accounts and ask sensible questions. You do not need to be an accountant; you need to be an intelligent reader (section 8 helps).
- **The strategy and the plan.** Where the hospital is trying to go, and the annual budget and business plan that get it there.
- **The Afya relationship.** What the partnership agreement commits each side to, and how we judge whether Afya is delivering.
- **The risks.** The main things that could go wrong, clinical, financial, operational, and reputational, and how they are being managed.
- **Your duties as a director.** The basics of what the law and good governance expect of you (section 9).

Nobody walks in with all of this. It is built through the induction, the board pack, the training programme, and simply sitting through a few cycles. Do not let the length of this list worry you; treat it as the syllabus for your first year, not a test you must pass tomorrow.

6.3 What good preparation looks like

The single habit that separates confident directors from anxious ones is **reading the pack before the meeting**. When the board pack arrives, set aside the time, read it, and note your questions. A director who has read the pack contributes well and feels in control. A director who has not spends the meeting lost. This one habit is most of the job.

A note on the anxiety this can cause. It is completely normal to look at the meetings, the reading, and the responsibility and feel it is a lot. It is a lot, and feeling that is a sign you are taking it seriously, which is exactly what good directors do. The workload becomes far more manageable once the rhythm is established, the templates are doing their job, and you have been through a cycle or two. The purpose of the enablement programme is to shorten that learning curve for you.

7. Chairing a committee for the first time

Several of you will chair or co-chair a committee. Chairing is a distinct skill from being a member, and it is very learnable. Here is what it actually involves.

What a committee chair does:

- **Sets the agenda** for each meeting, with the Company Secretary and the relevant executives, so the committee spends its time on what matters.
- **Runs the meeting:** keeps it to time, makes sure every member is heard, draws out the quiet voices, prevents any one person from dominating, and moves the group to clear decisions.
- **Reaches and records decisions:** makes sure the committee actually concludes, and that the decision and its reasons are minuted.
- **Reports to the board:** presents the committee's one-page report at the board meeting and flags what needs a board decision.
- **Manages conflicts:** ensures anyone with an interest declares it and steps out (section 10).



What a good chair is not: the chair is not the person who does all the work or knows all the answers. The chair *draws the work and the answers out of the committee and the management team*. Your job is to lead the conversation, not to be the expert on everything in it. Leaning on your members, on management, and on the Company Secretary is not weakness; it is exactly how chairing is done.

Co-chairing (as on the Finance, Investment and Operations Committee) simply means two chairs share the leadership, agree the order of business, and alternate or co-preside. Where co-chairs cannot agree on a matter, it goes up to the full board rather than being forced through. It is a deliberate way to share oversight fairly.

First-time chair's starting move: before your first meeting, sit down with the Company Secretary and your committee's terms of reference (already drafted), and walk through the agenda together. Thirty minutes of preparation with the Secretary will make your first meeting feel manageable.

8. Reading the numbers when you are not a finance person

Most directors are not accountants, and you do not need to be. You need to be a confident reader of a few things, and to know which questions to ask.

- **The management accounts** tell you how the hospital performed against its budget. Look first at where reality differs from plan (the "variances"), and ask why.
- **Cash and working capital** tell you whether the hospital can pay its way. A hospital can be profitable on paper and still short of cash, so this matters.
- **The dashboard** summarises performance on a page. Learn to scan it for what is off target.

Good questions a non-finance director can always ask: *Why is this different from what we planned? Is this a one-off or a trend? What is being done about it? What would have to be true for this to get worse?* You do not need to know the answer; asking the question is the governance. If a report is unclear, say so. If the numbers do not seem to add up, ask. An honest "I do not understand this, please explain it" is one of the most valuable things a director can say, and it improves the reporting for everyone.

9. Your duties and your conduct as a director

You do not need to be a lawyer, but you should know the shape of your duties. As a director you are expected to:

- **Act in good faith in the best interests of the company** as a whole.
- **Exercise care, skill, and diligence:** prepare, attend, pay attention, and apply your judgement.
- **Avoid and declare conflicts of interest** (section 10).
- **Keep board matters confidential.**
- **Act within your powers**, using the Delegation of Authority schedule as your guide to what is the board's to decide, what belongs to a committee or the Governance Committee, and what is management's.

In the meeting itself, a few habits make you effective: come prepared, listen as much as you speak, ask rather than assume, disagree on the substance without making it personal, and once a decision is properly made, get behind it. Confidentiality is not optional; what is said in the boardroom stays there.



10. Conflicts of interest and family interests

Because HSH is owned by two families, works with a partner (Afya), and takes advice from external parties, conflicts of interest will arise. That is normal. What matters is that they are handled openly. Handled well, they are a sign of a healthy board; hidden, they are a danger.

The rule is simple:

- **Declare it.** If a matter touches you, your family, or a business you are connected to, say so at the start of the discussion.
- **Step out of it.** Where the interest is material, leave the room for that decision so the others can decide freely. This is called *recusal*, and it protects both you and the board.
- **Record it.** The Company Secretary keeps a register of interests and notes recusals in the minutes.

This applies to everyone, including the independent directors and advisers. Declaring a limit on your own involvement is not a weakness; it is one of the clearest signs of a trustworthy director.

11. Disagreeing well: how a two-family board stays strong

You will not always agree, within a family or across the two families, and you are not meant to. A board where everyone always agrees is not governing; it is nodding. The strength of this board will come from how well it *disagrees*.

A few principles make disagreement productive rather than divisive:

- **Everyone is heard before a decision is made.** People accept outcomes they did not prefer far more readily when they had a genuine voice in the discussion. Giving each other that voice is the single most powerful thing this board can do for its own unity.
- **Reasons are explained.** When a decision goes one way, the reasoning is made clear, so those who disagreed understand *why*, even if they still would have chosen differently.
- **Decisions are for the company, not for a side.** Keep returning to the question, what is best for HSH? It is the common ground both families share.
- **Once decided, support it.** A decision properly reached, with everyone heard, is the board's decision, and the board stands behind it together.

This is not soft advice; it is how family-owned institutions survive across generations. The process by which you decide matters as much as the decisions themselves. A board that decides fairly earns the trust that lets it decide hard things.

12. The Afya relationship, from your seat

Afya runs the hospital's operations in partnership with HSH. As a director, your relationship to that partnership is one of oversight, and one point matters above all:

The board keeps its own, independent view of how Afya is performing. Because the Afya-HSH Governance Committee is run jointly with Afya, the board cannot rely on it alone to grade Afya's own delivery. That is why the board's committees (Audit and Risk on the numbers, Quality and Clinical Governance on care) form their own read, and why the full board reviews the partnership every quarter.



Your job is to make sure HSH always knows, independently, whether the partnership is delivering. That protects the hospital's ability to thrive with or without Afya, which is one of the board's four core jobs.

13. Common first-timer questions, answered plainly

"This feels like a lot of meetings and a lot to know. Is that normal?"

Yes, completely. It looks heaviest at the start, when everything is new and the model is being set up. Once the rhythm is established and you have been through a cycle, it settles into something manageable and, for most directors, genuinely rewarding. The training programme exists to get you there faster.

"I am not a finance or governance expert. Am I out of my depth?"

No. No single director holds every skill; that is the whole point of having a board of eight with different strengths. You were chosen for what you bring. You will learn the rest, and asking questions is not a weakness, it is the job.

"How much time will this really take?"

Plan for roughly one to two days a month of board and committee work once things are running, more in the early months and more if you chair a committee, all on top of your existing role. It is a real commitment, and a plannable one.

"What if I disagree with a decision, or with the other family?"

Say so, on the substance, in the room. Disagreement handled openly makes the board stronger. Once a decision is properly made with everyone heard, support it.

"Am I personally responsible if something goes wrong?"

Directors carry real duties, so take them seriously: prepare, attend, and apply your judgement in good faith. Directors who do that, and who declare their conflicts, are acting exactly as the law and good governance expect. The Company Secretary and the chair are there to keep you on the right side of your duties, and the board should arrange appropriate directors' insurance.

"What if I do not understand something in a meeting?"

Ask. Every time. "Please explain that" is one of the most useful sentences in any boardroom, and it improves the reporting for everyone.

14. Your first ninety days: a simple starter list

1. Read this guide and the Board Operating Model once, end to end.
2. Meet the Company Secretary; understand how the board pack, calendar, and minutes work.
3. Read your committee's terms of reference, and if you chair, walk the first agenda through with the Secretary.
4. Get the induction pack: the strategy, the latest accounts, the Afya agreement summary, and the risk register.
5. Attend the enablement and training sessions; they are built to teach exactly what this guide describes.
6. For your first board meeting, block the reading time, read the pack, and write down your questions.
7. When in doubt about who decides what, check the Delegation of Authority schedule.



15. A short glossary

- **Board / director.** The body that governs the hospital, and its members. You.
- **Executive director (ED).** A director who is also part of the executive or ownership of the company.
- **Non-executive director (NED).** A director who is not part of management, brought in for independent judgement.
- **Lead Independent Director (LID).** A senior independent director who balances the chair and can lead where the chair has a conflict.
- **Medical Director (MD).** The senior executive running the hospital's clinical operations; here, also an executive director.
- **Governance vs management.** Governing is setting direction and holding to account (the board's job); managing is running the hospital day to day (management and Afya's job).
- **Committee.** A smaller group of directors that examines an area (finance, audit, quality, people, transformation) in depth and reports to the board.
- **Board pack.** The consolidated set of papers circulated before each meeting.
- **Delegation of Authority (DOA).** The schedule that says which decisions belong to the board, to committees, to the Governance Committee, and to management.
- **Reserved matters.** The big decisions the board keeps for itself and does not delegate.
- **Reporting by exception.** Reporting that highlights what is off target rather than restating everything.
- **Recusal.** Stepping out of a decision in which you have a conflict of interest.
- **Fiduciary duty.** Your legal duty to act in the best interests of the company as a whole.
- **Afya-HSH Governance Committee.** The joint body, HSH and Afya together, that oversees the day-to-day operating partnership and reports up to the board.

Prepared by Dr Debo Odulana for the executive directors of Havana Specialist Hospital. Consult for Africa. Confidential. This guide is educational and supportive; it does not replace formal legal advice on directors' duties, which the board should obtain.